

Negative recommendation

- **Topical antibiotics as monotherapy** are not recommended for the treatment of mild to moderate papulopustular acne.
- **Artificial UV radiation** is not recommended for the treatment of mild to moderate papulopustular acne.
- The **fixed-dose combination of erythromycin and zinc** is not recommended for the treatment of mild to moderate papulopustular acne.
- **Systemic therapy with anti-androgens, antibiotics, and/or isotretinoin** is not recommended for the treatment of mild to moderate papulopustular acne.

Note: Limitations such as financial constraints, reimbursement policies, legal restrictions, drug availability, or licensing may necessitate the use of a lower-strength treatment as first-line therapy. Prescribers should be aware of the risk of antibiotic resistance when prescribing antibiotics.

Recommendations for severe papulopustular/moderate nodular acne

High strength of recommendation

- **Oral isotretinoin monotherapy** is strongly recommended.

Medium strength of recommendation

- **Systemic antibiotics**^{2,3} in combination with: - **adapalene**⁴, - the **fixed-dose combination of adapalene and benzoyl peroxide (BPO)**, or - **azelaic acid**⁵ can be recommended.

Low strength of recommendation

- **Systemic antibiotics**^{2,3} in combination with **BPO**⁵ can be considered.
- **For females:** Hormonal anti-androgens in combination with: - systemic antibiotic^{2,3} and topicals (excluding topical antibiotics), or - a topical treatment (excluding topical antibiotics) can be considered.

Open recommendation

- Due to insufficient evidence, no recommendation can be made for or against the use of **red light, IPL, laser, or photodynamic therapy (PDT)** in the treatment of severe papulopustular/moderate nodular acne.
- Although **PDT** is effective, a recommendation cannot be made due to lack of standardized treatment regimens and concerns about acute adverse reactions.

Negative recommendation

- **Single or combined topical monotherapy** is not recommended.
- **Oral antibiotics as monotherapy** are not recommended.
- **Oral anti-androgens as monotherapy** are not recommended.
- **Visible light as monotherapy** is not recommended.
- **Artificial UV radiation** is not recommended.

Note: Limitations such as financial resources, reimbursement, legal restrictions, availability, or drug licensing may affect treatment choices.

Prescribers should remain vigilant about the risk of antibiotic resistance.